

ADMISSION NOTE - Patient synth_05 - Acute exacerbation of COPD with type 2 respiratory failure

GANGA RAM CITY HOSPITAL, NEW DELHI
ADMISSION NOTE

MRN: 894551

Patient Name: Vijay Singh, 68/M

Admission Date: 14/07/26

Admitting Consultant: Dr. A. Sharma, Pulmonology

DIAGNOSIS:

1. Acute Exacerbation of Chronic Obstructive Pulmonary Disease (COPD)
2. Type 2 Respiratory Failure
3. Systemic Hypertension
4. Rule out Cor Pulmonale

CHIEF COMPLAINTS:

1. Increased breathlessness - 3 days
2. Cough with purulent sputum - 3 days
3. Drowsiness - 1 day

HISTORY OF PRESENT ILLNESS:

Patient is a known case of COPD for the past 10 years, on irregular treatment with inhalers. He developed worsening of his chronic breathlessness over the last 3 days, associated with an increase in cough productive of thick, yellowish sputum. He reports no relief from his usual inhaler (salbutamol). For the past 1 day, his family members noticed he has been increasingly drowsy and not responding as usual. No history of fever, chest pain, palpitations, or pedal edema. Bowel and bladder habits are normal.

PAST HISTORY:

- Known case of COPD for 10 years.
- Known case of Hypertension for 15 years, on regular medication.
- No known history of Diabetes Mellitus, Coronary Artery Disease, or Thyroid disorder.
- History of smoking 1 pack of cigarettes daily for 30 years, quit 5 years ago.
- No history of previous surgeries.

PERSONAL HISTORY:

- Diet: Mixed, appetite reduced for 3 days.
- Sleep: Disturbed due to breathlessness.
- Smoker: Ex-smoker.
- Alcohol: Occasional.

GENERAL EXAMINATION:

Patient is drowsy but arousable to verbal commands. Appears tired and in respiratory distress. Central cyanosis present. No pallor, icterus, clubbing, lymphadenopathy or pedal edema.

Vitals: PR 110/min, regular. BP 150/90 mmHg. RR 28/min, shallow, using accessory muscles. SpO2 86% on Room Air. Temp 98.6°F afebrile. GCS: E4 V4 M6 (14/15).

SYSTEMIC EXAMINATION:

- RESPIRATORY SYSTEM: Bilateral symmetrical air entry, diffuse polyphonic rhonchi and wheezes heard over both lung fields. Prolonged expiratory phase.
- CARDIOVASCULAR SYSTEM: S1, S2 heard. No murmurs.
- PER ABDOMEN: Soft, non-tender. No organomegaly.
- CENTRAL NERVOUS SYSTEM: Drowsy, no focal neurological deficit.

INVESTIGATIONS SENT:

- CBC, KFT, LFT, Serum Electrolytes
- Arterial Blood Gas (ABG)
- C-Reactive Protein (CRP)

- Chest X-Ray PA view
- Sputum for Gram Stain, Culture & Sensitivity
- ECG

Reports Awaited.

PLAN:

- Admit to ICU for monitoring and management.
- Start Non-Invasive Ventilation (BiPAP).
- Start IV antibiotics, steroids, and bronchodilators.
- Monitor vitals, SpO2, and sensorium.

PROGRESS NOTE ? Day 1 - Patient synth_05 - Acute exacerbation of COPD with type 2 respiratory failure

GANGA RAM CITY HOSPITAL, NEW DELHI
PROGRESS NOTE

Date: 15/07/26

Patient Name: Vijay Singh, 68/M

Subjective:

Patient feels marginally better. Breathlessness has reduced but still present. Drowsiness has improved significantly. Tolerating BiPAP well.

Objective:

General Condition: Conscious, oriented. Less tachypneic. Cyanosis resolved.

Vitals: PR 96/min, BP 140/88 mmHg, RR 24/min, SpO2 91% on BiPAP (FiO2 30%), GCS 15/15.

Resp: Air entry improved. Bilateral rhonchi still present, but intensity reduced.

Investigations:

- ABG (on BiPAP): pH 7.35, pCO2 60 mmHg, pO2 75 mmHg, HCO3 29. Shows improvement in hypercapnia.
- CBC: Hb 14.2 g/dL, TLC 14,500/cumm, Platelets 2.1 Lacs/cumm.
- CRP: 48 mg/L.
- CXR: Hyperinflated lung fields, flattened diaphragm. No acute consolidation.
- Sputum C/S: Report awaited.

CURRENT MEDICATIONS:

1. INJ MONOCEF 1g IV 1-0-1
2. INJ SOLU-MEDROL 40mg IV 1-1-1
3. NEB DUOLIN (Levosabutamol + Ipratropium) 1 respule Q6H
4. NEB BUDECORT (Budesonide) 1 respule 1-0-1
5. TAB TELMA 40mg 1-0-0
6. TAB AMLOKIND 5mg 0-0-1
7. TAB PAN-40 1-0-0

PLAN:

- Continue current management.
- Attempt to wean from BiPAP, with periods on nasal prong O2.
- Encourage chest physiotherapy and deep breathing exercises.
- Monitor vitals, ABG, and clinical status.

PROGRESS NOTE ? Day 2 - Patient synth_05 - Acute exacerbation of COPD with type 2 respiratory failure

GANGA RAM CITY HOSPITAL, NEW DELHI
PROGRESS NOTE

Date: 16/07/26

Patient Name: Vijay Singh, 68/M

Subjective:

Patient feels much better. Breathlessness is minimal at rest. Cough has reduced. Good oral intake.

Objective:

General Condition: Alert, comfortable. No respiratory distress at rest.

Vitals: PR 84/min, BP 130/80 mmHg, RR 20/min, SpO2 94% on 2L O2 via nasal prongs. GCS 15/15.

Resp: Air entry good. Few scattered rhonchi present.

Investigations:

- Repeat CRP: 22 mg/L.

CURRENT MEDICATIONS:

1. INJ MONOCEF 1g IV 1-0-1
2. INJ SOLU-MEDROL 40mg IV 1-1-1
3. NEB DUOLIN 1 respule SOS
4. NEB BUDECORT 1 respule 1-0-1
5. TAB TELMA 40mg 1-0-0
6. TAB AMLOKIND 5mg 0-0-1
7. TAB PAN-40 1-0-0

PLAN:

- Patient has shown significant clinical improvement.
- He has been successfully weaned off NIV and is stable on low-flow oxygen.
- Plan for discharge today evening.
- Convert IV medications to oral regimen.
- Counsel patient and family regarding medication adherence, correct inhaler technique, and warning signs of another exacerbation.
- Reinforce importance of smoking cessation.

LABORATORY REPORT - Patient synth_05 - Acute exacerbation of COPD with type 2 respiratory failure

GANGA RAM CITY HOSPITAL - LABORATORY REPORT
Patient Name: Vijay Singh, 68/M, MRN: 894551

COMPLETE BLOOD COUNT (Sample: 14/07/26)
PARAMETER VALUE UNITS REFERENCE RANGE

Hemoglobin 14.2 g/dL 13.0 - 17.0
Total Leucocyte Count 14,500 /cumm 4,000 - 11,000
Platelet Count 2.1 Lacs/cumm 1.5 - 4.5

RENAL FUNCTION TEST (Sample: 14/07/26)

Blood Urea 35 mg/dL 15 - 40
Serum Creatinine 1.1 mg/dL 0.7 - 1.3

ARTERIAL BLOOD GAS (Sample: 14/07/26, on Room Air)

pH 7.28 - 7.35 - 7.45
pCO2 75 mmHg 35 - 45
pO2 54 mmHg 80 - 100
HCO3 28 mEq/L 22 - 28

INFLAMMATORY MARKERS

C-Reactive Protein (14/07/26) 48 mg/L (< 6.0)
C-Reactive Protein (16/07/26) 22 mg/L (< 6.0)

MICROBIOLOGY

Sputum Culture & Sensitivity (Sample: 14/07/26) Report Awaited.

MEDICATION RECORD - Patient synth_05 - Acute exacerbation of COPD with type 2 respiratory failure

GANGA RAM CITY HOSPITAL, NEW DELHI
MEDICATION RECORD

Patient Name: Vijay Singh, 68/M, MRN: 894551

ADMISSION MEDICATIONS (Started on 14/07/26):

1. INJ MONOCEF 1g, IV, 1-0-1
2. INJ SOLU-MEDROL 40mg, IV, 1-1-1
3. NEB DUOLIN, via nebulizer, 1-1-1-1 (Q6H)
4. NEB BUDECORT, via nebulizer, 1-0-1
5. TAB TELMA 40mg, Oral, 1-0-0
6. TAB AMLOKIND 5mg, Oral, 0-0-1
7. TAB PAN-40 40mg, Oral, 1-0-0

DISCHARGE MEDICATIONS (Prescribed on 16/07/26):

1. TAB AUGMENTIN 625mg, Oral, 1-0-1 for 5 days
2. TAB WYSOLONE 30mg, Oral, 1-0-0 for 5 days then taper
3. TAB DERIPHYLLIN RETARD 150mg, Oral, 1-0-1
4. Formoterol+Budesonide Inhaler, 2 puffs, 1-0-1
5. Levosalbutamol Inhaler, 2 puffs, SOS for breathlessness
6. TAB TELMA 40mg, Oral, 1-0-0 (to continue)
7. TAB AMLOKIND 5mg, Oral, 0-0-1 (to continue)
8. TAB PAN-40 40mg, Oral, 1-0-0 (for 1 week)

DISCHARGE ADVICE - Patient synth_05 - Acute exacerbation of COPD with type 2 respiratory failure

GANGA RAM CITY HOSPITAL, NEW DELHI
DISCHARGE SUMMARY

MRN: 894551
Patient Name: Vijay Singh, 68/M
Date of Admission: 14/07/26
Date of Discharge: 16/07/26

- DIAGNOSIS:
- 1. Acute Exacerbation of Chronic Obstructive Pulmonary Disease
 - 2. Type 2 Respiratory Failure (Resolved)
 - 3. Systemic Hypertension
 - 4. Cor Pulmonale

COURSE IN THE HOSPITAL:

Mr. Vijay Singh, a known case of COPD and Hypertension, was admitted with acute breathlessness and drowsiness. He was diagnosed with an acute exacerbation of COPD with Type 2 Respiratory Failure. He was managed in the ICU with Non-Invasive Ventilation, IV antibiotics, steroids, and bronchodilators. His condition improved significantly, he was weaned off NIV and is now stable on minimal oxygen support. He is being discharged in a stable condition.

CONDITION AT DISCHARGE:

Patient is conscious, alert, and oriented. Hemodynamically stable. Maintaining SpO2 of 94% on 2L/min O2 via nasal prongs, to be continued at home. Advised to procure an oxygen concentrator. Accepting oral diet and medications.

- ADVICE ON DISCHARGE:
- | No. | Medication Name | Dosage | Frequency | Duration |
|-----|-------------------------------|---------|-----------|-------------|
| 1. | TAB AUGMENTIN | 625mg | 1-0-1 | 5 Days |
| 2. | TAB WYSOLONE | 30mg | 1-0-0 | 5 Days* |
| 3. | TAB DERIPHYLLIN RETARD | 150mg | 1-0-1 | 2 Weeks |
| 4. | Formoterol+Budesonide Inhaler | 2 puffs | 1-0-1 | To Continue |
| 5. | Levosalbutamol Inhaler | 2 puffs | SOS | To Continue |
| 6. | TAB TELMA | 40mg | 1-0-0 | To Continue |
| 7. | TAB AMLOKIND | 5mg | 0-0-1 | To Continue |
| 8. | TAB PAN-40 | 40mg | 1-0-0 | 1 Week |

*Taper TAB WYSOLONE to 20mg for 5 days, then 10mg for 5 days, then stop.

DIET: Normal diet. Avoid cold food/drinks.
ACTIVITY: Rest for 1 week. Avoid strenuous activity and exposure to dust/smoke.

- FOLLOW-UP INSTRUCTIONS:
- Follow up in Pulmonology OPD with Dr. A. Sharma after 7 days.
 - In case of increased breathlessness, fever, or drowsiness, report to the emergency room immediately.
 - Learn and practice correct inhaler technique.

PENDING RESULTS:

- Sputum Culture & Sensitivity Report. Please collect from the lab after 2 days and show during follow-up.